



Human Relations
[0018-7267(200207)55:7]
Volume 55(7): 861–885: 024544
Copyright © 2002
The Tavistock Institute ®
SAGE Publications
London, Thousand Oaks CA,
New Delhi

Sensations, bodies, and the ‘spirit of a place’: Aesthetics in residential organizations for the elderly

Patricia Yancey Martin

ABSTRACT

This article uses data collected in the 1970s and 1980s from old people's homes (OPHs) in the United Kingdom to explore how aesthetics are organized in residential organizations for the elderly. The analysis reviews the sensations to which OPH members are subjected and reveals the role of power in organizing aesthetic experiences. Paradoxes associated with framing residential organizations as homes and with controlling residents' bodies are explored. I focus on the senses of smell, sight, touch, and sound with attention to staff and residents' views of residents' bodily messes and their constructions of residents' death and dying. In exploring the 'spirit of a place,' I differentiate homey from institutional facilities (as ideal types) by noting their physical, cultural, and social attributes. In a section on socially constructing residents' bodies, I explore how homey (more than institutional) OPHs use power to enhance residents' dignity and rights. Conclusions affirm the value of placing the ethnographer and her bodily sensations inside the story and the need for more research on smells and power's role in creating a positive spirit in residential organizations for the elderly.

KEYWORDS

aesthetic judgments ■ ambiance ■ 'bodily messes' ■ body politics
ethnography ■ institutional vs homey 'spirit'

Over 20 years ago, I did an ethnographic study of small residential organizations in England that included old people's homes (as residential facilities for the elderly were then called), children's homes, and probation hostels (Martin, 1984). I wanted to find out if these places resemble Erving Goffman's 'total institutions.' Do they, for example, totally control the lives of residents who are partly shut off from but also partly embedded in the community? As I progressed, my focus embraced other topics such as the role of trade unions in these organizations (Martin, 1984). Yet, regardless of my focus, I always maintained a positivist stance. *I left myself – my aesthetic judgments and bodily sensations – out of the story.* Positivist principles at the time told ethnographers to embrace objectivity and neutrality and to avoid subjectivity and personal judgment in reporting 'results.' Even though my bodily sensations were essential to accomplishing the fieldwork (see Goodall, 1989; Stoller, 1997), my written reports made no mention of them. I left myself out of the story even when my notes showed I disliked some facilities so much I had to force myself to return to them after an initial visit.

When I participated in a workshop on organizing aesthetics at the Villa Certosa di Pontignano near Siena (Italy) in May 2000, I recalled that project. The presentations made me reflect on my aesthetic experiences with many different organizations but especially residential facilities for the elderly. I recalled many repulsive but also uplifting sensations associated with my fieldwork in those organizations and, thus, I decided to revisit my notes to *view* the residents and staff, and myself, from an aesthetics perspective. What could I learn about their members', and my, aesthetic experiences and judgments?¹

This article reports on that exercise. When organizations serve as a residence for people – 24 hours a day, seven days a week, year-round – the sights, smells, and sounds of eating, cooking, bathing, toileting, laundering, housecleaning, and human conflict are pervasive. The organizations I visited were no exception. They barraged me with sensate experiences to which, as my notes show, I (often strongly) reacted. As I reviewed my notes to explore the sights, smells, touch, and sounds of old people's homes, I saw the centrality of bodies in their operations (Conquergood, 1991). I also saw how emotions, and lack of emotion, affected residents and staff. The old people's homes I visited appeared to have been conceptualized, organized, and operated with the *bodies* of elderly people in mind. These facilities cannot be understood, in my view, unless the social relations and dynamics of 'body politics' – issues of power and control in the management of bodies – are addressed. How power is deployed, and by whom, affects the aesthetic character of these organizations by influencing the 'spirit of a place,' as I discovered.

My field notes show that my informants and I had aesthetic experiences

and made aesthetic judgments using multiple and often contradictory standards. As a rule, residents liked having privileges and freedom but they also liked orderliness and being waited upon by staff. They liked staff who treated them respectfully and created a warm 'emotional climate' (Barbalet, 1998). Staff liked some residents more than others, for example, they disliked those who were dependent or recalcitrant and they resisted seeing, smelling, and touching residents' bodily waste (see Gubrium, 1975). I had aesthetic experiences and made judgments too. For example, I *liked* facilities that 'smelled good,' 'looked colorful and cheerful,' and had 'happy' residents who made eye contact with me and I *disliked* those that were unfriendly, excessively concerned with cleanliness and order, or where staff were 'cheeky' (or smart-alecky) with residents. In sum, residents, staff, and I made a variety of aesthetic judgments, some of which we felt passionately about.

In this report, I use the phrase the 'spirit of a place' to capture the corpus of sensory perceptions in and reactions to residential organizations for the elderly (Strati, 1990).² This phrase refers to an organization's ambiance and *emotional climate* (Barbalet, 1998: 159; defined below), including its members' and my sensate and emotional reactions to the physical and social context. It entails *tacit knowledge* which is an aspect of an organization's spirit that is incapable of being verbally expressed (Polanyi, 1962, 1966) but is nevertheless perceived, known, and reacted to by members. As subsequent material shows, the spirit of a place is produced both by the unreflective practices and reflexive praxis of intentional actors (see Arnett & Arneson, 1999 on reflective practice versus reflexive praxis). I address the role of power in organizing aesthetics and understanding aesthetic experiences and judgments. Succeeding sections contend that members experienced and communicated the spirit of a place in how they treated each other, dressed and smelled, decorated rooms and hallways, cleaned and maintained buildings and grounds, reacted to residents' bodily failures, and in other ways. They also suggest that nastiness in old people's homes – 'bodily messes,' death, social conflict – is (in part) a reflection of organizational framing, policies, and practices (Strati, 1999: 102–3).

Organizational aesthetics, according to Strati (1999: 1) requires (a) 'a sensory faculty and experience' for example someone experiencing smell, touch, hearing, or sight; and (b) '... a reaction to the sensory experience,' for example, an intentional judgment of the experience. While such experiences and judgments are ultimately the purview of individuals, we can obtain clues to the *nature of an organization's aesthetic character* by paying attention to its members' aesthetic reactions. Thus, besides my own aesthetic experiences and judgments, I take into account the aesthetic experiences and judgments of residents, staff, and administrators. This exercise

thus takes place on two levels. One is reacting to my own sensory perceptions, and two is my gut-level aesthetic response and judgment about old people's homes relative to the dignity and autonomy of the residents. These two levels concern different organizational artefacts (Strati, 1999: 158), e.g. bodily emissions on the one hand and pictures and personal effects in occupied social spaces on the other. According to Strati (1999: 158), '[B]y mean of artefacts, organizational cultures exert control over their members and educate their perceptive faculties – that is, their "sense of taste, of smell, of touch, of hearing, as well as sight" (following Gagliardi, 1996: 573).'

Subsequent sections show that aesthetic experiences in some facilities are far more extreme or forceful than in others. All old people's homes provoke profound aesthetic experiences – of smell, sight, sound, and tactile sensations – and they have highly dependent residents. Unlike many other settings, residents cannot go home at night nor can they easily switch organizations. Bodily experiences associated with dependence, physical and mental decline, and death and dying are endemic. Yet, despite these conditions, old people's homes vary in 'spirit.' Differences occur (a) because aesthetic judgments are highly personalized and individual and (b) because the social organization of social relations and implementation of authority and control – or power – vary. Members' morale and the impact of their aesthetic judgments on morale are affected by both (Diamond, 1992). My analysis suggests that emphasizing residents' humanity and dignity and using social spaces, personal touches, and organizational policies to promote (or not) residents' pleasure, happiness, and autonomy are the means by which aesthetic experience is organized in these facilities.

This exercise adds to understanding of aesthetics in residential organizations for the elderly in several ways. First, it shows how 'bodily messes' (see below) are aesthetically experienced and judged by residents, staff/administrators, and outsiders, including the ethnographer, and it makes evident the role of power in framing aesthetic judgments. Residents' bodies are subjected to the organization's authority and justificational ideology that place (into rooms, beds), control (residents' bodies, time, activities, privileges), and frame/define them (as able or dependent) in particular ways (see Martin, 1997 on justificational accounts). Residential organizations deal with elderly residents' diminishing capacities – relative to mobility, waste elimination, disease, and mental functioning – in ways that residents experience as alternatively demeaning or empowering. Constrained by lack of control over their bodies and bodily functions, many residents depend on staff for help with basic functions – such as eating, bathing, walking, and eliminating waste – and to preserve their feelings of self-worth. However, residents also exercise

agency via making demands on staff, resisting authority, and insisting on certain rights and privileges.

As a second contribution, I show that old people's homes deal with the loss of bodily functions and death in varying ways, with some fostering an impersonal and sterile spirit and others a warmer, 'homey' spirit. I argue that the spirit of a place is actively created through the dynamic of human agency. Although issues of bodily waste, decline, and death permeate these organizations, they do not inevitably produce negative aesthetic experiences and judgments. Minimally endowed facilities, my notes show, can be rendered 'beautiful' by the intentional practices (or theory-informed praxis; see Arnett & Arneson, 1999) of administrators, staff, and residents (Ramirez, 1991).

Third, this effort seeks to counteract overly rationalist, mind-focused, bureaucratic views of residential organizations for the elderly (Strati, 1996, 1999; Calás & Smircich, 1997). By bringing sensate and emotional experiences to the fore, it shows what residential organizations *look, smell, sound, and feel like* to residents, staff, and (one) ethnographer. Such an account adds flesh and bones to so-called objective depictions based on staff-client ratio, quality of care, and 'time spent with residents' indicators and illuminates what living and working in old people's homes actually 'feels' like.

Succeeding sections review the context and methods of my earlier fieldwork and describe the goal of placing myself inside the story. I explain the concept of body politics and review the paradox of calling residential organizations 'homes.' I report on the sights, smells, touch, and sounds of old people's homes relative to bodily messes and death and dying and discuss the spirit of a place relative to power and organizing aesthetics. In the discussion, I note how residents' bodies are disciplined by power (Foucault, 1977) in homey versus institutional facilities and assess the significance of aesthetic experiences in these settings.

Background, context, procedures

Between 1979 and 1985, I conducted participant observation and open-ended interviews in 16 old people's homes (hereafter OPHs) in a county in the south-west of England. I did the original fieldwork in January–April 1979 and returned in 1980, 1981, 1982, and 1985 to several of the sites. I immersed myself – mind, body, and emotions (see Stoller, 1997) – in the activities of the OPHs by observing and experiencing their routines, e.g. at mealtimes, waking and going to bed, and bathing time (Martin, 1984). I interviewed directors and other staff and observed and engaged residents in informal conversations about activities, routines, plans, and feelings. I sat

with residents and staff in common areas, ate meals with them, observed meal preparation, serving, and clean-up, and other activities (e.g. staff meetings). I interviewed government and private foundation officials who monitored the facilities and I accompanied government officials on site visits.

In accord with the call by anthropologists and sociologists to reinstate the body and the sensual in ethnographic work (Goodall, 1989; Stoller, 1989, 1997; Conquergood, 1991; Fineman, 1993; Rasmussen, 1999), I place myself inside the story. Following procedures advocated by Strati, I use 'fragments' from my field notes to gain 'empathic understanding of intentional action' (1999: 67) relative to organizational aesthetics. I use my personal experiences and judgments and my imagination and reflexive thought (or introspection; see Ellis, 1991) to place myself in the position of residents (and, to a lesser extent, staff) in order to understand their aesthetic experiences and judgments (Strati, 1999: 67–73).³ In this report, both people and organizations are referred to by pseudonyms.

Body politics and the home paradox

In revisiting my notes, I came to see the body as a contested, political site rather than a natural, neutral site (Foucault, 1977; Davis, 1997). Old people's homes are made nasty or nice, homey or institutional (see below), not (only) because residents have physical (and mental) problems but also because they are characterized by power dynamics. Administrators and staff use power to define and treat residents in particular ways (Lukes, 1974). *Body politics* refers to power struggles over the body, including how they are negotiated (Lorber, 2001). How are residents' bodies represented and treated in old people's homes? Who decides? Who says what residents' bodies are or should be like, whether they are beautiful and ugly? Who makes bodies go and stay, do, or desist from doing? In the OPHs I visited, administrators and staff exercised extensive power over residents. However, residents had power too, particularly in facilities where administrators used their power to enhance residents' agency (see below). Residents also resisted control in some facilities, e.g. by refusing to be compliant.

Foucault's (1977) synopsis of systems that *discipline* bodies for various ends – from creating soldiers to students or factory workers – helped me understand how administrators and staff 'distribute, monitor, and control' residents' bodies. Judith Lorber (1994: 40) says 'physical bodies are always social bodies,' building on Robert Connell's thesis that, 'The body without ceasing to be the body is taken in hand and transformed in social practice' (1987: 83). The *physical bodies* of residents in old people's homes become

social bodies when staff and administrators talk about them, physically touch them, assign them to rooms, allow or forbid them to go for walks, and allow (or not) residents to control their medications (Foucault, 1977; Bourdieu, 1990; Davis, 1997; Barbalet, 1998). Guided by administrators and the facility's philosophy, routines, social relations, and cultural ideology, I saw OPH staff socially constructing residents' bodies through talk and practice (Bordo, 1993). They *enacted* a conception of bodies – as strong or weak, able or disabled, touchable or untouchable, clean or dirty, fair or foul smelling – in ways that shaped residents' perceptions, experiences, and feelings.

Residents occupy OPHs around the clock, participating in activities such as eating/feeding, clothing, bathing, sleeping, eliminating waste, recreating, socializing, making friends, having arguments, smoking cigarettes, drinking alcohol, and so on. Yet they lack sufficient power to make staff do their bidding. Often residents cannot control their own bodies relative to place, time, activity, or function. For example, they cannot go for a stroll at will, make a cup of tea at 2 a.m., or walk the dog at sunrise, as they could 'at home.' Some cannot move from their beds or use the toilet without help from staff. They are subjected to the power and discipline of a formal organization that manages their use of space, social relations, behavior, and bodily functions (Lukes, 1974; Foucault, 1977: Part III, Ch. I). The way administrators, staff, and external overseers label, manage, supervise, and monitor residents' bodies fosters a particular 'spirit of a place,' as I discuss later.

Home or workplace?

Related to body politics dynamics is the question of whether old people's homes are true *homes*. In a true home, one has privacy, autonomy, the right to live in squalor, or stay up all night. Like homes, residential facilities are corporeal structures with bedrooms, toilets and baths, kitchens, laundries, sitting areas, dining rooms, and storage spaces. Yet, '*old people's homes*' have many artefacts that are not very home-like, e.g. strangers living side by side, sounds of anguish and suffering, alienation, extreme dependence, and the certainty that you only leave to die (perhaps stopping by hospital on the way). Residential organizations may be *like a home* but, as many scholars note, *they are not homes* in the usual sense (Diamond, 1992). They are formally administered organizations with budgets, paid staff, trade unions, and structured mealtimes, bathing routines, and rules about coming, going, using the kitchen, and taking medication (Gubrium, 1975; Martin, 1984; Foner, 1994; Willson, 1996; Mathey et al., 2001).

OPH administrators use the term 'home' in paradoxical ways. When they depict a facility as a home, they imply that residents have the same rights

and privileges as they would 'at home.' But this claim is false. Residents cannot have pets. Their privacy is minimal to nonexistent; personal activities like bathing, toileting, dressing, and eating are publicly administered and observed. Sex is so taboo that residents who make an issue of it are stigmatized (Foner, 1994). Old people's homes are workplaces for the staff – sites of employment with specific hours, duties, routines, and holidays. They are, in contrast, the residents' only abode. The disjunction between the label and the reality of residents' formally administered lives is not lost on residents who move to such a facility when they can no longer manage on their own (Diamond, 1992). Moreover, the modal resident stays until she dies.⁴ A British report on the county I studied says 55 percent of residents leave UK 'homes' by dying and 34 percent exit to hospital, never to return.

The following quotation from an administrator, writing to potential residents of Marblehead House (a pseudonym), shows this paradox. While telling residents they are moving to a new home/house where they will have new companions, he acknowledges that they will lose privacy and be subjected to 'necessary routines.' He acknowledges that a move into Marblehead House marks a break with their former life – friends, family, physician, and interests – and the start of a new one that is formally administered. The letter implicitly admits that control over their lives will pass from their hands into his and those of the corporate interests that Marblehead House represents.

Entry to the home [*sic*] would mean that your way of life will change considerably. Sharing your house [*sic*] with a number of other people inevitably means the loss of some privacy but provides the compensation of companionship. This also brings with it the opportunity to help others, either as individuals, or through helping as you can around the house. You should try to retain an active contact with your present friends and keep all your existing interests outside the home . . . If you live near to Nixon [town] and your present doctor is willing, it will be possible for you to remain with him [*sic*]. When needed he will visit you at Marblehead [name of facility] if you are unable to attend his surgery. Minor ailments can be nursed in the home but if you should need to enter hospital, your place in Marblehead will be kept for your return for a period of up to eight weeks and occasionally longer . . . As in any well-ordered home, some routine is necessary but I hope that you will regard such routine as for your convenience. No routines are inflexible and there are no fixed bed times.

Contradicting the last sentence, staff at Marblehead told me residents are in bed by 9.00 p.m., although 'they *can* keep their lights on as late as they like,'

suggesting that lights out, not bedtime, is the actual choice. The claim that 'old people's homes' are true homes is contradicted when 'necessary routines' dictated by others set the parameters of everyday life.

Sensate experience: Smells, sights, touch, sounds

Residents and staff in OPHs are subjected to a maze of smells, sights, touches, and sounds that prompt aesthetic reactions. In this section, I review sensate experiences that my notes suggest were acute for the people I studied or for myself. For example, all of us viewed bladder incontinence as producing smells and touch that we defined as repugnant. When mentally diminished residents cried out in pain or confusion, residents (and staff) felt emotional distress. As Stoller (1989) notes, sight can be contained within walls but smell and sound cannot (also Corbin, 1986; Goodall, 1989; Classen et al., 1994; Rasmussen, 1999). Smells waft from one person or room to another and sounds can permeate an entire building. Disagreements characterized by shouting or name-calling upset their hearers as well as participants. I was upset by staff bickering in two facilities and, from my observations, residents were as well. Whereas the next section on the spirit of a place focuses on the aesthetics of social relations, I focus here on the sensations of smells, sights, touch, and sounds.

As if it were yesterday, I remember how Marblehead House smelled. My notes read, 'The smell of disinfectant is *very* strong and pervasive.' I noted my effort to avoid grimacing or holding my nose at the harsh smell of cleaning fluid that permeated the building. 'My eyes watered as my heart went out to residents (and staff) who had to breathe this smell continuously,' I wrote. The director Mr Avery (a pseudonym), a 40-year-old nurse by profession, was concerned to assure a high standard of 'cleanliness' in the setting and the upshot was heavy reliance on disinfectants. Smells of food being cooked, medicine, laundry being done, and urine are usual in residential organizations for the elderly (see Diamond, 1992). Perhaps Mr Avery used disinfectant to camouflage such scents. Apparently, he failed to consider that the smell of cleaning fluids can also be revolting. In violating the 'ideal of olfactory purity' that is normative for organizations (Classen et al., 1994: 175), Mr Avery created a repulsive aesthetic experience.⁵

Bodily messes

Bodily messes, a phrase used by Diamond (1992) in discussing US nursing homes, refers to OPH residents' bodily limits and attenuation. Many residents

have dementia, some are in wheelchairs, many cannot dress, bathe, eat, or use the toilet without assistance. Incontinent residents must be cleaned and, to accomplish this, staff must touch their soiled bodies and clothing. Residents' bodies, even when small in stature, can be heavy and staff have difficulty lifting and moving them.

Both residents and staff make aesthetic judgments about incontinence. When a bedridden incontinent resident rings a buzzer to request help with the toilet, staff are likely to find she has soiled herself unless they respond immediately. A four-minute task of helping her out of bed, onto the toilet, and back into bed turns into a 20-minute task, often taking two workers, of getting her out of bed, washing urine (or feces) from her body, helping her don clean clothes, replacing soiled sheets or pads, and helping her back to bed. Because of fear of AIDS and other diseases, and a distaste for touching bodily waste, staff wear rubber gloves to remove soiled clothing, wash urine or feces from residents' bodies, and help with elimination tasks. And they view these tasks as repulsive. Some incontinent residents view their own lack of bladder control as repugnant and are embarrassed to ask for help. The premium placed by western societies on whole, healthy, able, clean, and good smelling bodies affects the aesthetic judgments of staff and residents alike (Lorber, 1994; Lorber & Martin, 2000).

My notes describe the embarrassment an elderly woman felt when she could not control the impulse to urinate:

By the time her brain signaled the need to urinate, she urinated nearly immediately; she could not hold back. She smiled in a shy way and said 'I'm sorry' to the attendant who came to her rescue. The attendant reassured her but she remained apologetic. The attendant said, 'Oh, you can't help it; don't feel bad. It's OK.'

Being unable to perform this function in private, without soiling herself or her environment, humiliated her. In needing a stranger's help to accomplish a personal, private task, her independence and dignity were compromised.

Residents' incontinence poses problems for staff. Most see it as repulsive and time-consuming, an unsavoury task that entails seeing, touching, and smelling soiled bodies, clothing, and linens. They believe it exposes them to germs or disease. Staff in 'institutional' OPHs (see below) often delayed responding to a resident's call for help and, when they did, treated her/him coolly or rudely. Furthermore, some staff talk about residents as if they are not present, treat them like children, or treat them as if their incontinence is intentional.

Residents disliked incontinent residents also, according to Mrs Jones at Bardwell House. She said: '[A]n incontinent person is smelly and messy and it disgusts and upsets the other residents . . . and takes away time of staff from doing other . . . more important things.' I too felt distaste in encounters with elderly residents who smelled unwashed or like urine. My notes described an elderly man whom I felt 'glad to see' but didn't want to 'come too close': 'Mr Watson is so bright and charming; his eyes light up when he sees me. But I almost hold my breath if he comes close. Why do they let him go around [smelling] like this?' When many residents are incontinent, a facility can smell like urine, a condition that staff, residents, and visitors find repulsive. Such a condition may have prompted Mr Avery to use strong cleaning fluids at Marblehead House.

Gubrium (1975) reported two decades ago in a study of US nursing homes that staff are routinely required to perform tasks that they view as repugnant and even disgusting (also Diamond, 1992; Lyman, 1993; Rae, 1998). He described the removal of impacted feces from extremely constipated residents; staff don plastic gloves and reach inside the resident's rectum to 'gouge' out hardened, dried feces, bit by bit (see Ellis, 1995). Staff hate this task relative to the sights, smells, and touch it entails, Gubrium says, and their distaste often transfers to the residents, diminishing their show of respect and attentiveness.

Death and dying

Death and dying are ever-present aspects of OPHs (Diamond, 1992). Death can be more or less traumatic, depending on how administrators and staff frame it and encourage residents to react. Even grief associated with dying is, as Glaser and Straus showed years ago (1964, 1965), socially constructed. This section focuses on how the death of some residents is experienced and judged aesthetically by OPH staff and residents.

Relative to death, Mr Avery said, 'We lose someone' about every five to six weeks; after saying this, he and his deputy laughed in an embarrassed way. Residents are 'the worst about acting like nothing has happened,' he explained. He told how one man reacted when he announced just before a meal that a resident had died: 'I announced [at mealtime] that Mrs T had died and one of the men muttered, "Who cares? I'm hungry; let's eat."' My notes show my dismay at his lack of awareness of the likely impact of his behavior on residents. I imagined how I would feel upon sitting down for a meal only to have someone announce that a friend had died. My notes read: 'This would not help me eat in peace . . . and the poor taste of doing this would irritate me with the speaker's gross insensitivity.'

A staff member at Bardwell House talked about residents' dying similarly. Mrs Foster said:

When residents come here, they stay until they die. We know it and they know it [that they die there] and it makes life tough. It seems like we make too little fuss over death, as if we fail to give it its due. But we *have to* [emphasis hers], for the other residents' sake.

My notes indicate a suspicion that institutional peace and calm as much as concern for residents' well-being may drive the down-playing of death. My notes read, 'It [down-playing death] causes less uproar among patients [*sic*] if it [death] is passed over lightly and it is less explosive to the relatives of the residents.'

The spirit of a place: Aesthetic judgments and power in OPHs

Both repugnance and beauty are socially constructed (Ramirez, 1991; Pelzer, 2000; Weiskopf, 2000), a dynamic that is no less true in OPHs than other contexts (Leidner, 1999). Aesthetic judgments are not inherent in the thing itself but are subject to interpretation. Plain, drab buildings do not automatically repulse just as affluent, ornate buildings do not automatically please. Bodily decline and failure are only prompts for aesthetic judgments, not guarantees of a particular outcome (Strati, 1996, 1998, 1999, 2000). Thus, while nearly everyone saw incontinence as 'smelly' and repulsive, some framed it as more a health problem over which residents lacked control whereas others blamed residents for a seeming moral failure of will. Staff in the latter category were often sullen and rude to residents (and each other).

The spirit of a place, the focus of this section, is a form of organizational knowledge that reflects a facility's culture and *emotional climate* relative to social relations, practices, routines, and tacit understandings. According to Barbalet:

Emotional climates are sets of emotions or feelings which are not only shared by groups of individuals implicated in common social structures and processes but which are also significant in the formation and maintenance of political and social identities and collective behavior. Emotional climate . . . includes emotional tones and patterns which differentiate social groups or categories by virtue of the fact that they are shared by their members and unlikely to be shared with non-members. (1998: 159)

Based on my observations of cultural artefacts, social relations, and routine practices, I identify two ideal types of facilities relative to their emotional climate, artefacts, and social relations; the two types are 'homey' and 'institutional' (see below). No facility was totally homey or institutional but all were generally closer to one or the other, with most closer to the institutional type. I judged how pleasing the facilities were to me and to residents and staff (as I interpreted them) in order to gain empathic understanding (Strati, 1999).⁶

Homey vs institutional?

'Homey' OPHs had an emotional climate that was friendly and welcoming. Staff and residents made eye contact and treated each other kindly and respectfully; they seemed glad to see me when I arrived. Relative to the physical setting, common areas were colorfully decorated, e.g. with brightly colored table-cloths in the dining room and soft, bright colors of paint on the walls. Sitting areas where family members visited or residents read and watched television were cluttered with residents' personal effects such as eye-glasses, books, shawls, and house slippers. In some facilities, residents lounged, conversed, or played games in common areas and moved freely around the facility. Some residents participated in future-oriented activities, e.g. planning outings and fetes or writing to the local government about having a 'pub' on site. Residents at Bardwell House took turns answering the telephone and played recorded music over an intercom system in the afternoons. When I arrived, they greeted me with smiles and sometimes offered me a cup of tea. In general, residents in homey facilities were friendly and curious rather than withdrawn or indifferent, as they often were at institutional sites. The staff in homey OPHs seldom wore uniforms thus I could not readily distinguish them, except by age, from residents. I agree with the assessment of Mrs Jones at Bardwell House who said, 'You have to have a happy staff or residents are unhappy too.' Her comment implies that staff members' feelings and emotions shape residents' feelings and emotions, a conclusion with which I agree.

Noisy and bustling homier OPHs contrasted with the preternatural quiet and lack of resident activity in institutional OPHs. Although similar to homey ones physically and relative to formal organization (residents in bedrooms by twos or fours, common eating and dining areas, etc.), institutional OPHs were less attractive visually and socially. Dining areas were less colorful and staff seemed less interested in the residents as individuals. Residents moved around less and even visitors were closely monitored. For example, one facility required all visitors to ring a bell and sign a guest book; it required me to

explain the purpose of my visit to an administrator, even upon repeat visits. My notes from an OPH with a bell say this, 'I was surprised. I had to press a buzzer to be let in, unlike most homes [I've visited].' In more institutional facilities, residents stayed in their rooms much of the day, away from common areas. If out of their rooms, they often sat in hallways with 'lap rugs' over their knees, neither reading nor talking. Few personal effects of residents were in evidence in common areas. When residents visited common areas, they tended to watch television rather than 'visit with' each other. Some institutional facilities reeked of cleaning fluid, as described earlier.

Staff in some institutional settings wore uniforms associated with their jobs, e.g. 'domestics' who cleaned toilets and floors wore one color and 'carers' who helped residents eat, dress, toilet, and bathe wore another, differentiating them from each other and from residents. When staff dressed this way, the facility seemed to have more interpersonal conflict although, of course, greater conflict could have preceded as well as followed the use of uniforms (see Martin, 1984). An institutional facility looked and 'felt' more like a formally administered organization than a 'home' in being formal, orderly, and distant rather than casual, cluttered, and emotionally close.

Socially constructing and controlling residents' bodies

In this final section, I explore how OPHs socially construct and control residents. Knowledge about social relations in OPHs can show how power *produces* aesthetic experiences and judgments (Foucault, 1977). In this section, I explore how social relations in OPHs were affected by staff and resident reactions to bodily messes and to death and dying. In doing this, I focus on the way power is organized (Lukes, 1974) including its role in constructing residents' bodies and in fostering practices that place a priority on (or downplay) their dignity and humanity.

Relations between OPH staff and residents are often conflictual. David Harris, a social worker, describes the stresses on staff of residential care including its 'nasty' aesthetics:

It is my contention that residential work is often nasty, uncomfortable, and unpleasant. Of course it *can* be a satisfying, positive and fulfilling experience but to deny the reality of the nastiness of residential care either to oneself or to others is to do a disservice to the people who are carers or who are cared for. New staff members are often surprised by the stressful, demanding nature of the work . . .

(Harris, 1979: 33)

What is nasty about 'residential care'? Residential work entails a '24/7' (24 hours a day, seven days a week) convergence of bodies, sensory perceptions, and emotional responses to bodies and sensory perceptions. While nastiness has many faces, Harris focuses on staff conflicts and resident abuse associated with residents' dependence, disruptive behavior, and mental incapacitation in a confined setting. [Diamond \(1992\)](#) talks about how administrative demands receive priority over residents' needs and Foner (1994) describes the pressures that unruly, mentally disabled, or angry residents bring to bear.

Some administrators create nastiness when they enact policies that detract from the residents' quality of life. For example, Mr Avery at Marblehead House designated one room as a 'sick room.' When he showed it to me, my surprise prompted him to say, 'Oh, it's not *officially* a sick room; but when people get very sick, we move them here . . . to protect other patients [from their disease].' I felt badly for residents who knew about the sick room and, upon being placed in it, likely assumed the worst. A county official told me later that residents who were moved to the 'sick room' often died and the other residents knew this. I saw designation of a sick room as a regrettable emphasis on decline and death instead of health and living. In contrast, Mrs Jones encouraged her Bardwell House residents to plan parties, outings, and other activities that are future-oriented.

The bodies of OPH residents are a material entity that staff/administrators place/locate (e.g. in particular rooms), regiment/manage (monitor as to dress, sleeping, etc.), and control (prohibit from cooking, taking walks, leaving, etc.; Davis, 1995, 1997). The facility's culture constitutes residents as texts through discourse by specifying their potential, limits, skills, and rights and orienting staff to negotiate residents' bodies via interactions and struggles over what they can do, where they must or must not go, and so on (Davis, 1997; Lorber, 2001). Some facilities define residents' bodies as having agency, capable of doing for and taking care of themselves whereas others focus on residents' lack of agency and dependence. In Foucault's terms (1977), the latter emphasize surveillance and articulate a 'discourse of knowledge' to frame elderly bodies as declining and in need of vigilant monitoring.

In general, homey OPHs enhanced residents' dignity by giving them more autonomy. Compared to institutional homes, they allowed residents to 'take risks' like they would take at home. For example, Mrs Jones of Bardwell House let (some but not all) residents control their medications rather than have staff administer them, even though she feared they would take the wrong quantity or drug. She allowed residents to go for walks and do other activities, as they would do at home.⁷

It's our job to make this place as much like a home as possible. Residents can come and go as they please, have people in, go out with them, go for walks, go drinking, and so on. It's *their* home [emphasis hers]. . . and it's our job to see that they come first, not the needs of staff and not the rules of Bardwell House.

Mrs Jones forbade staff to stop residents from 'wandering off,' even when they were at risk of becoming lost:

We do not go and bring them back. Even the confused ones can go and come as they please. If they want us to walk with them, then we try to. But under no circumstances are my staff to go up to Mr Smith and say, 'All right, now, let's go back.' That's Mr Smith's business. If someone gets lost or confused, we'll look for them but it's their right to go for a walk anytime they feel like it or want to.

Bardwell House gave residents other privileges such as use of the kitchen during non-meal times. It encouraged communication and support among residents, resulting in friendships and even romantic attachments among some. Mrs Jones described the positive social relations among Bardwell residents:

They help each other. . . they help each other get around, remind them about things . . . They try to cheer each other up. We have a romance going on now. A little blind woman and a man whose appearance is grotesque. He has cancer in his head that has required all kinds of surgery. Each day, a community nurse comes by to change his bandage. His face is very ugly plus he wears this bandage plus he has a hole in the roof of his mouth, so he's hard to understand when he talks. Because of this romance, though, he talks more and [he's] become easier to understand. They [the man and women] are very sweet.

She acknowledged that the residents' bodies were flawed and, in many ways, unattractive; the woman in the story was blind and the man suffered from cancer and a deforming surgery and had difficulty speaking. Yet she saw them as adults with a right to have romantic attachments. Mrs Jones, furthermore, *enforced* respect for residents through the rules she made for staff.

In more institutional OPHs, relations among residents and between staff and residents were often discordant. Mr Caldwell, director of Naples Residential Home, worried when staff used 'bad language' to residents and residents to staff (see [Gabriel, 1998](#)). He once overheard a staff member having

difficulty with an overweight 'female resident' mutter as the resident walked away, 'You old cow!' He 'forbids' such talk, he said, but it occurs anyway.

I forbid [*sic*] this [use of insults, taunts] but it's hard to control, especially since some residents abuse [*sic*] staff as well . . . One resident had a chest cold and wasn't supposed to lie flat . . . was supposed to move around . . . go to the bathroom when she had to go and so on. Well, the lady was sick and she didn't want to move about. So Betty Price [the third in charge] would make [or force] her to get up and go to the bathroom, whether she was agreeable to it or not. She [the ill resident] really resented this . . . and would say all kinds of abusive [*sic*] things.

When staff prevent residents from taking risks, they construct residents' bodies as needing to be controlled, as subject to harm if they act independently. In many respects, such assumptions are correct since OPH residents are often not strong or independent. However, staff in institutional OPHs tend to control residents' movements and activities even when it is in the residents' best interest to decide or act for themselves. When staff do everything for residents, the residents become passive, accustomed to being 'done for' and hesitant to act on their own. In short, they become dependent. If staff then ask them to behave independently, to do more for themselves, they often resist (see Martin & Segal, 1977).

Mrs Blakely, director of Lofland House, described such a development during a crisis when many staff were out sick with the flu. She asked the residents to pitch in and help with the work of the facility but, Mrs Blakely said, the residents refused.

One man said, 'I'm not going to help you. That's what we *pay* you for.' He made it very clear he expected to be waited on and not have to do anything to make the home a cleaner or better place to live. I worry about him because he encourages negative attitudes in other residents who otherwise wouldn't have them.

Lofland House was beset with other conflicts between director and staff. When Mrs Blakely tried to 'clamp down' on staff who were failing to do their jobs properly, those at fault stayed out of work rather than submit to her authority. The remaining staff became unhappy due to their increased workload and Mrs Blakely's failure to discipline the recalcitrant co-workers. Residents picked up the staff members' discontent and became upset too. As a result, Lofland House developed a sulky and inhospitable emotional

climate. This story supports Mrs Jones' claim that happy residents require a happy staff.

Staff in more institutional facilities say it is 'too much work' or 'not worth the bother' to ask residents to 'do for themselves.' For example, requiring residents to make their own beds, serve food at mealtimes, or clear away dishes after a meal requires them to explain, encourage, demonstrate, remind, support – and be patient. These tasks take time and 'doing for' is faster and easier. Wives and mothers who go home at day's end to prepare meals for their own families told me they do not want to 'waste time' urging residents to 'serve' themselves or clear away mealtime dishes. If they serve the evening meal (or 'tea') at 4 p.m., they can get home to their families sooner. Even though the residents' next meal is not until 8 a.m. the next morning, and knowing that 16 hours between meals is a very long time, Lofland House staff resisted Mrs Blakely's urging to serve the meal later. As this example shows, administrative ineffectiveness, and the individual and group dynamics that accompany it, can create a negative emotional climate and related aesthetic experiences.

The interpersonal dynamics of such establishments are ironic. Staff members who resent residents' dependence also resist helping them be independent. Unless administrators use their power to insist on residents' agency and rights as true adults, and unless they require staff to support this view, staff are likely to favor ease and speed over the challenge of requiring residents to exercise agency and take (more) responsibility for themselves.

Discussion and conclusions

My Villa Certosa di Pontignano experience and this exercise have increased my understanding of the dynamics of aesthetics, particularly the role of human agency in creating a positive 'spirit' in residential organizations. In exploring my own and others' aesthetic experiences and judgments, I conclude that official tasks, job descriptions, and mission statements (among other formal qualities) are only part of the story. Sensory experiences, judgments, and emotional reactions permeate the social relations and practices of residential organizations for the elderly. Representations that omit them are partial at best and misleading or false at worst (see Fineman, 1993; Putnam & Mumby, 1993; Martin & Knopoff 1997; Calás & Smircich, 1997; Leidner, 1999; Pierce, 1999; Wharton, 1999; Campbell, 2001; Martin, 2001). This exercise increases my appreciation of power's role in shaping aesthetics.

The centrality of bodies in old people's homes requires a methodology that takes bodies, bodily sensations, bodily messes, and body politics centrally

into account. As Stoller says, '... discussions of the sensuous body require sensuous scholarship in which writers tack between the analytical and the sensible, in which embodied form as well as disembodied logic constitute scholarly argument' (1997: xv). Stoller's argument affirms my decision to position myself 'inside' rather than 'outside' the story (see Gherardi, 2002 on positioning). Acknowledging how my experiences shaped my aesthetic judgments and representations of old people's homes makes them visible for readers to assess. From outside the story (and written text; Ellis, 1995), I characterized OPHs as if they were a certain way, irrespective of my experiences and judgments, by fixing them in time and space. From inside the story and written into the text, I show here that these settings are not only the way I experienced (or wrote about) them, nor are they only the way residents or staff experienced them (see Goodall, 1989). They are all of those ways, and more.

Stoller emphasizes ethnographers' need to be sensually immersed in a setting to truly comprehend it:

Comprehension demands the presence, not the absence, of the ethnographer ... It demands ... that ethnographers open themselves to others and absorb their worlds. Such is the meaning of embodiment. For ethnographers embodiment is ... the realization that ... we too are consumed by the sensual world, that ethnographic things capture us through our bodies, that profound lessons are learned when sharp pains streak up our legs in the middle of the night.

(1997: 23)

And, I might add, when residents' bodies, staff sullenness, and cleaning fluids repulse.

Many people who have read this article recalled smells from a visit to an old people's (or nursing) home. One said, 'That's what it's like; that's how it is. It *smells* ... like cabbage [cooking] or something strong ... like you say.' People who visit a family member may be anxious about whether their loved one is being well cared for or whether s/he should be there at all and their senses may be heightened, including the sense of smell. Perhaps anxiety associated with my research heightened my sense of smell. Was I unusual in hating the stench of cleaning fluids? After I experienced that odor and judged it repulsive, everything I experienced thereafter was affected; I could not ignore it. I fought an impulse to hold my nose or leave and I experienced a surge of empathy for residents in wondering how they could have an appetite or enjoy a meal. Greater attention to experience(s) associated with smells and aromas in residential and other organizations would be useful because, as

Classen et al. (1994) note, current norms require organizations to be 'silent' on smell. When this norm is violated, is an organization judged as aesthetically flawed? (See Strati, 2000 on smell's uniqueness; also Corbin, 1986; Classen et al., 1994; Rasmussen, 1999.)

This exercise prompts three conclusions. One, it suggests that old people's homes are rich sites for studying aesthetics. An encounter with a residential organization for the elderly – whether by residents, administrators, staff, external authorities, family members, or ethnographers – assures a barrage of sensations that prompt aesthetic judgments. Their sights, smells, sounds, touch, and emotional dynamics are extensive and weighty. Few organizations provide greater variety or stronger aesthetic experiences.

Two, the analysis reveals how power shapes aesthetic experiences and judgments and shows that power's role in body politics, the creation of artefacts, and the enforcement of standards for interaction is key to understanding old people's homes. The collective actions of administrators, staff, and residents create artefacts, practices, routines, and values that foster differing kinds of 'spirit' in these organizations. All OPHs have features that are difficult to manage, e.g. 'bodily messes,' death and dying, limited financial resources, residents with declining health, and unequal power among residents and staff. Yet many features are susceptible to intentional action, e.g. whether they frame residents as active or passive, how they use common areas, how they decorate the facility (materials, colors, etc.), whether they let residents control their medications, mobility, and schedules. Power's role in effecting the variable features of OPHs interests me most. When administrators use the organization's authority to prioritize residents' autonomy and rights, they show that power can be used to create a positive spirit.

In discussing organizations that discipline bodies, Foucault argues that:

[E]ven if they [those with power] do not make use of violent or bloody punishment, even when they use 'lenient' methods involving confinement or correction, it is always the body that is at issue – the body and its forces, their utility and their docility, their distribution and their submission . . . [T]he body is . . . directly involved in a political field; power relations have an immediate hold upon it; they invest it, mark it, train it, torture it, force it to carry out tasks, to perform ceremonies, to emit signs.

(Foucault, 1977: 25–6)

Pertinent to this study, OPH residents' bodies are, without question, 'directly involved in a political field.' Their bodies are invested, marked, trained, subjected to ceremonies, and required to emit signs. These practices

need not harm OPH residents, however, or take advantage of their vulnerability.

My analysis suggests that a homey OPH uses power over residents leniently and compassionately, rather than harshly or stringently, to promote residents' autonomy and rights. When administrators go 'around staff' to affirm residents' rights to make choices and control (aspects of) their lives, they enact a transfer that shifts power from staff to residents (see Leidner, 1993 on power dynamics in interactive service work). Administrators who do this foster a healthy climate for residents and create a positive spirit overall.

A third conclusion addresses policy implications. My analysis suggests that aesthetics should be taken into account when planning and running residential organizations for the elderly. A recent study in the Netherlands documents the 'healthiness' of an aesthetically pleasing nursing home. The authors found that 'an improved ambiance' relative to the physical environment (including atmosphere of the dining room) and organization of 'nursing staff assistance' produced improvements in residents' nutritional and health statuses in the course of a year, while residents' health in a control group facility declined (Mathey et al., 2001). If some aesthetic experiences are health-enhancing, others presumably have the opposite effect. The Dutch study, along with mine, suggests that an attractive physical setting along with affirmative social relations and a theoretically informed conception of residents' abilities and rights can create a pleasing and materially beneficial 'spirit' in residential organizations for the elderly. With the world's frail elderly increasing at a very high rate, those who care about their quality of life may want to take these issues these into account (Myles & Quadagno, 2000).

Acknowledgements

I thank Antonio Strati and Pierre Guillet de Montoux for organizing the Villa Certosa di Pontignano workshop on 'Organizing Aesthetics' and for their assistance and that of three anonymous *Human Relations* reviewers. I thank Judith Lorber, Jennifer Pierce, Andrea Willson, Jill Quadagno, Debra Street, David Collinson, Margaret Collinson, and especially John Reynolds for their editorial advice. I also thank the residents and staff of 16 UK old people's homes for their help.

Notes

- 1 In the 1980s, I noted that the British public and media supported children's homes more than old people's homes and conjectured that perhaps the public had more positive views of children whose lives are before them than the elderly whose lives

- were nearing an end (Martin, 1984). Perhaps fears about growing old or disgust over how the elderly look, act, or smell accounts for this situation (see Barbalet, 1998 on fear).
- 2 This phrase comes from one of Antonio Strati's interviewees about how he uses 'signals' conveyed by how people dress and speak to discover the 'spirit of a place' (Strati, 1990: 209).
 - 3 My knowledge of OPH aesthetics would be severely limited if not for residents and staff who shared their experiences and judgments. Many experiences and judgments of residents differed from those of staff and administrators' and from my own. Given this complexity, understanding OPHs' aesthetic character required me to take multiple standpoints and related experiences into account. Following feminist standpoint theory (Harding, 1991), I examined OPHs from my standpoint – as a relatively distant, removed scholar – and from the (respective) standpoints of residents, staff, administrators, and various external authorities. I used a double- (triple? quadruple?) reflexivity to extend the research beyond myself to take *multiple standpoints* into account. One aim of this study was to represent the experiences in/of OPHs in a 'less false' way (Harding, 1991; also Martin, 2001; Martin et al., 2002).
 - 4 Eighty-five percent of UK old people's home residents in the late 1970s to mid-1980s were women, a percentage that has persisted or increased in both England and elsewhere. Due to this situation, I refer to elderly residents as women (see Gattuso & Bevan, 2000).
 - 5 Unclean, unconventionally dressed, obese, disabled, tattooed and pierced bodies are often frowned upon and sanctioned (Bordo, 1993). Even security or cleaning staff must wear clean, neat clothing, with conventionally styled hair and minimal jewelry and makeup or risk being sanctioned.
 - 6 I do not mean to imply that the 'spirit' of any OPH was totally homey or institutional aesthetically nor that all residents (or staff) preferred homey facilities. As I say in the text, no single facility was as good or bad as these two ideal types. It is my observation, however, that residents in homey OPHs were more active, alert, and engaged and they (and the staff) were definitely friendlier to and more interested in me. Since I view social engagement as a sign of health, I would choose a homey over an institutional facility for anyone I care about.
 - 7 Mrs Jones (and staff) did not allow residents who had short-term memory problems or other mental incapacitation to control their medications. Furthermore, even though she said all residents, even those who are 'confused,' could take walks whenever they chose, I did not observe residents leaving the building to take a walk during my visits. I suspect informal social norms prevented residents from taking walks alone even when those in authority said it was allowed.

References

- Arnett, R.C. & Arneson, P. *Dialogic civility in a cynical age: Community, hope, and interpersonal relationships*. Albany: State University of New York Press, 1999.
- Barbalet, J.M. *Emotion, social theory, and social structure: A macrosociological approach*. Cambridge: Cambridge University Press, 1998.
- Bordo, S. *Unbearable weight: Feminism, western culture, and the body*. Berkeley: University of California Press, 1993.
- Bourdieu, P. *The logic of practice*. Palo Alto, CA: Stanford University Press, 1990.
- Calás, M.B. & Smircich, L. Predicando la moral en calzoncillos? Feminist inquiries into business ethics. In A.L. Larson & R.E. Freeman (Eds), *Women's studies and business ethics*. New York & Oxford: Oxford University Press, 1997, pp. 50–79.

- Campbell, R. *Emotionally involved: Research on rape as emotional labor*. London & New York: Routledge, 2001.
- Classen, C., Howes, D. & Synnott, A. *Aroma: The cultural history of smell*. London & New York: Routledge, 1994.
- Connell, R. *Gender and power: Society, the person, and sexual politics*. Cambridge: Polity Press, 1987.
- Conquergood, D. Rethinking ethnography: Towards a critical cultural politics. *Communication Monographs*, 1991, 58(June), 179–94.
- Corbin, A. *The foul and the fragrant: Odor and the French social imagination*. Cambridge, MA: Harvard University Press, 1986.
- Davis, K. *Reshaping the female body: The dilemma of cosmetic surgery*. New York: Routledge, 1995.
- Davis, K. *Embodied practices*. London: Sage, 1997.
- Diamond, T. *Making gray gold: Narratives of nursing home care*. Chicago, IL: University of Chicago Press, 1992.
- Ellis, C. Sociological introspection and emotional experience. *Symbolic Interaction*, 1991, 14(1), 23–50.
- Ellis, C. *Final negotiations: A story of love, loss, and chronic illness*. Philadelphia, PA: Temple University Press, 1995.
- Fineman, S. Organizations as emotional arenas. In S. Fineman (Ed.), *Emotions in organizations*. London: Sage, 1993, pp. 9–35.
- Foner, N. *The caregiving dilemma: Work in an American nursing home*. Berkeley: University of California Press, 1994.
- Foucault, M. *Discipline and punish: The birth of the prison*. Trans. A. Sheridan. New York: Pantheon Books, 1977.
- Gabriel, Y. An introduction to the social psychology of insults in organizations. *Human Relations*, 1998, 51(11), 1329–54.
- Gagliardi, P. Exploring the aesthetic side of organizational life. In S.R. Clegg, C. Hardy & W.R. Nord (Eds), *Handbook of organization studies*. London: Sage, 1996, pp. 565–80.
- Gattuso, S. & Bevan, C. Mother, daughter, patient, nurse: Women's emotion work in aged care. *Journal of Advanced Nursing*, 2000, 31(4), 892–9.
- Gherardi, S. Feminist theory and organizational theory: A dialogue on new bases. In C. Knudsen & H. Tsoukas (Eds), *Organizational theory as science: Prospects and limitations*. Oxford: Oxford University Press, 2002.
- Glaser, B.G. & Straus, A.L. The social loss of dying patients. *American Journal of Nursing*, 1964, 64(June), 119–22.
- Glaser, B.G. & Straus, A.L. *Awareness of dying*. Chicago, IL: Aldine, 1965.
- Goodall, H.L. Jr *Casing a promised land: The autobiography of an organizational detective as cultural ethnographer*. Carbondale & Edwardsville: Southern Illinois University Press, 1989.
- Gubrium, J.F. *Living and dying at Murray Manor*. New York: St Martin's Press, 1975.
- Harding, S. *Whose science? Whose knowledge? Thinking from women's lives*. Ithaca, NY: Cornell University Press, 1991.
- Harris, D. The nastiness of residential care. *Social Work Today*, 1979, 10, 33.
- Leidner, R. *Fast food, fast talk: Service work and the routinization of everyday life*. Berkeley: University of California Press, 1993.
- Leidner, R. Emotions in interactive service work. *The Annals of the American Academy of Political and Social Science*, 1999, 561(January), 81–95.
- Lorber, J. *Paradoxes of gender*. New Haven, CT: Yale University Press, 1994.
- Lorber, J. *Gender inequality: Feminist theories and politics*, 2nd edn. Los Angeles, CA: Roxbury, 2001.
- Lorber, J. & Martin, P.Y. The socially constructed body: Insights from feminist theory. In

- P. Kvisto (Ed.), *Illuminating social life: Classical and contemporary theory revisited*, 2nd edn. Thousand Oaks, CA: Pine Forge, 2000, pp. 183–206.
- Lukes, S. *Power: A radical view*. London: Macmillan, 1974.
- Lyman, K.A. *Day in, day out with Alzheimer's: Stress in caregiving relationships*. Temple: Temple University Press, 1993.
- Martin, J. & Knopoff, K. The gendered implications of apparently gender-neutral theory: Re-reading Max Weber. In A.L. Larson & R.E. Freeman (Eds), *Women's studies and business ethics*. New York & Oxford: Oxford University Press, 1997, pp. 30–49.
- Martin, P.Y. Trade unions, conflict, and the nature of work in residential organizations. *Organization Studies*, 1984, 5(2), 169–85.
- Martin, P.Y. Gender, accounts, and rape processing work. *Social Problems*, 1997, 44(4), 464–82.
- Martin, P.Y. 'Mobilizing masculinities': Women's experiences of men at work. *Organization*, 2001, 8, 587–618.
- Martin, P.Y. & Segal, B. Bureaucracy, size and staff expectations for client behavior. *Journal of Health and Social Behavior*, 1977, 18(December), 376–90.
- Martin, P.Y., Reynolds, J. & Keith, S. Gender bias and feminist consciousness among judges and lawyers: A standpoint theory analysis. *Signs: Journal of Women in Culture and Society*, 2002, 27(3).
- Mathey, M.F.A.M., Vanneste, V.G.G., de Graaf, C., de Groot, L.C.P.G.M. & van Staveren, W.A. Health effect of improved meal ambience in a Dutch nursing home: A one-year intervention study. *Preventive Medicine*, 2001, 32(5), 416–23.
- Myles, J. & Quadagno, J. Envisioning a *Third Way*: The welfare state in the twenty-first century. *Contemporary Sociology*, 2000, 29(1), 156–67.
- Pelzer, P. Disgust in organizations. Paper presented at the Workshop on Organising Aesthetics, Siena, Certosa di Pontignano, May 2000.
- Pierce, J. Emotional labor among paralegals. *The Annals of the American Academy of Political and Social Science*, 1999, 561(January), 127–42.
- Polanyi, M. *Personal knowledge*. London: Routledge, 1962.
- Polanyi, M. *The tacit dimension*. Garden City, NY: Doubleday, 1966.
- Putnam, L.L. & Mumby, D.K. Organizations, emotions, and the myth of rationality. In S. Fineman (Ed.), *Emotions in organizations*. London: Sage, 1993, pp. 36–57.
- Rae, H.M. Managing feelings: Caregiving as emotion work. *Research on Aging*, 1998, 20(1), 137–60.
- Ramirez, R. *The beauty of social organization*. Munich: Accedo, 1991.
- Rasmussen, S.J. Making better 'scents' in anthropology: Aroma in Taureg socioculture systems and the shaping of ethnography. *Anthropological Quarterly*, 1999, 72(2), 55–73.
- Stoller, P. *The taste of ethnographic things: The senses in anthropology*. Philadelphia: University of Pennsylvania Press, 1989.
- Stoller, P. *Sensuous scholarship*. Philadelphia: University of Pennsylvania Press, 1997.
- Strati, A. Aesthetics and organizational skill. In B.A. Turner (Ed.), *Organizational symbolism*. London: Sage, 1990, pp. 208–22.
- Strati, A. Organizations viewed through the lens of aesthetics. *Organization*, 1996, 3(2), 209–18.
- Strati, A. Organizational symbolism as a social construction: A perspective from the sociology of knowledge. *Human Relations*, 1998, 51(11), 1379–402.
- Strati, A. *Organization and aesthetics*. London: Sage, 1999.
- Strati, A. The aesthetic approach in organization studies. In S. Linstead & H. Höpfl (Eds), *The aesthetics of organization*. London: Sage, 2000, pp. 13–34.
- Weiskopf, R. Organizing aesthetics: (De)constructing the story of 'the iron cage' – towards an aesthetics of folding. Paper presented at the Workshop on Organising Aesthetics, Siena, Certosa di Pontignano, May 2000.

- Wharton, A. The psychosocial consequences of emotional labor. *The Annals of the American Academy of Political and Social Science*, 1999, 561(January), 158–76.
- Willson, A. When nursing the elderly doesn't end at work: Caregivers' narratives in the paid and unpaid spheres. Unpublished Master's thesis, Department of Sociology, Virginia Polytechnic Institute and State University, Blacksburg, VA, 1996.

Patricia Yancey Martin is Daisy Parker Flory Professor of Sociology at Florida State University where she teaches organizations, gender, and qualitative methods. She edited a book, *Feminist organizations: Harvest of the new women's movement*, with Myra Marx Ferree, in 1995. Her current work concerns masculinity/ies in corporations (*Organization*, 2001), cross-national research on organizations (*Gender, Work, & Organization*, 2002), and gender in the legal institution (*Signs*, 2002).
[E-mail: pmartin@coss.fsu.edu]